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PROVIDER EDITION - Focused Action Report (Sample)

Session-Ready Strategies for Riley T.

A clinician-facing report on a single presenting challenge - with a graded, session-by-session plan you can put into practice this week.

FOCUS AREA

Transition-driven shutdowns

PREPARED FOR (CLIENT)	SERVICE	ENGAGEMENT
Riley T., Age 9	**Pediatric Occupational Therapy**	**45 minutes, weekly**

Sample report. This is a demonstration built on fictional sample data to show partner providers the format and depth of a Sponsored Access report. Every recommendation is derived from the (sample) questionnaire answers shown in the appendix.

PROVIDER ENGAGEMENT SNAPSHOT

How this report was scoped to your service

These details are answered by the **provider** on the engagement information form and shape every recommendation that follows - so the plan fits the way you actually work, not a generic template.

Service type	Pediatric Occupational Therapy
Session format	1:1, in-clinic
Session length & cadence	45 minutes, weekly
Setting	Outpatient clinic (sensory gym + table-work room)
Engagement goal	Increase tolerance for non-preferred and novel table-top tasks and reduce shutdown during in-session transitions.

Clinical summary

Riley presents with transition-driven shutdowns: at the moment of moving onto a non-preferred or novel task, verbal output drops, the body stills, and engagement ceases. The pattern is best understood as an anxiety-driven regulation response to uncertainty rather than non-compliance, which is why predictability - not increased demand - is the lever. This report translates the family's questionnaire answers into a session-ready plan: establish a previewable session architecture, target the transition moment with signaling and bounded choice, then grade in novelty one small step at a time, with a clear de-escalation protocol for in-session shutdowns.

CLINICIAN QUICK REFERENCE

The one-page read on Riley

Designed to sit beside you in session, or to hand to a covering clinician. Everything here is drawn from the questionnaire answers.

THE PATTERN IN ONE LINE	Riley shuts down at the moment of transition to a non-preferred or novel task. The shutdown is a regulation response to uncertainty, not non-compliance.
WHAT IT LOOKS LIKE IN SESSION	Sharp drop in verbal output, body stilling, averted gaze, gripping a preferred object. Onset is the transition itself, not the task.
WHAT REDUCES IT	Preview the transition (visual + verbal first-then). Offer a bounded choice. Insert proprioceptive input before the demand. Keep the session opening identical each week.
WHAT EXTENDS IT	Repeating or escalating the demand, adding time pressure, narrating your own frustration, or removing the preferred item abruptly.
EARLY-WARNING SIGNS	Slower responses, shorter answers, increased object-focus, a glance toward the door. Intervene here - before the full shutdown - for the best result.
RE-ENTRY AFTER A SHUTDOWN	Lower demand to near-zero, restore predictability, offer regulating input, then re-approach the SAME task in a smaller step. Do not debrief in the moment.
SESSION-END SUCCESS LOOKS LIKE	Riley tolerated one previewed transition with a bounded choice and re-engaged within ~2 minutes of a wobble - without a full shutdown.

SESSION CHECKLIST

Your 45 minutes, previewed

Tick as you go. Built from the family's answers and scoped to your engagement form (1:1, in-clinic, 45 minutes, weekly).

Before Riley arrives (2 minutes)

- [ ] Skim the At-a-Glance page and the last Session Log row.
- [ ] Visual schedule of today's 45 minutes printed / on the board.
- [ ] First-then board within reach for every planned transition.
- [ ] Sensory-gym option (wall push, ball, weighted lap pad) staged.

First 5 minutes — the opening ritual

- [ ] Same greeting as last week — the constant is the point.
- [ ] Known proprioceptive warm-up BEFORE any request.
- [ ] Walk the visual schedule together; no task demands yet.

At every transition

- [ ] Preview ~2 minutes out — verbal + visual cue, never a cold switch.
- [ ] Offer a bounded choice of two (material, order, or seat).
- [ ] Proprioceptive input immediately BEFORE the non-preferred task.
- [ ] Watch the early-warning window: slowed responses, shorter answers, object-focus — intervene there, not at the full shutdown.

Wrap-up (last 5 minutes)

- [ ] End on a known, tolerable task — never on the hardest demand.
- [ ] Preview next week in one sentence.
- [ ] Log one line: state at open, what worked, flag for home.

IN-SESSION DE-ESCALATION

If a shutdown happens mid-session

A step-order protocol. The throughline: reduce demand, restore predictability, offer regulation, re-approach smaller.

Step 1 - Reduce demand to near-zero
Stop issuing the instruction. Restating it extends the shutdown. Signal with your body that the demand is paused.

Step 2 - Restore predictability
Quietly name what is happening and what comes next: "We're going to wait. Then we'll pick." Predictability is the regulator here.

Step 3 - Offer regulating input, do not require it
Present a known proprioceptive or deep-pressure option (wall push, weighted lap pad, ball squeeze). Offer; never force.

Step 4 - Re-approach in a smaller step
Once verbal output and posture return, re-enter the SAME task at its smallest first step with a bounded choice. Do not jump ahead.

If shutdown deepens or distress escalates
Lower all demand, reduce sensory load (lighting, noise, proximity), and shift to a regulating activity. End on a known, tolerable task. Debrief with the caregiver after session, not with the child in the moment.

TRIGGERS → WHAT WORKS

The match list

Reported triggers on the left; the response the family's data supports on the right. Scan before the session; act at the early-warning signs.

KNOWN TRIGGER	WHAT WORKS IN SESSION
Unsignaled transition off a preferred task	First-then structure + a ~2-minute previewed switch. Never cold.
Novel materials with no preview	Preview it, then present only the smallest first step.
Perceived evaluation ("let's see if you can…")	Reframe as a bounded choice: "start with blue or green?"
Time pressure ("hurry, come on")	Drop the clock. Keep the sequence — "first this, then we pick."
Sensory load late in the session	Proprioceptive input before the demand; reduce lighting/noise.
Preferred object removed to force a switch	No specific counter reported — let the object ride along; lower demand and re-approach smaller.

IN-SESSION LANGUAGE

What to say at the transition

The wording that regulates versus the wording that extends the shutdown - matched to this client's triggers.

SAY THIS	INSTEAD OF
"First the puzzle, then we choose the next thing together."	"Okay, all done - new activity now." (unsignaled switch)
"In two minutes we'll move. I'll show you what's next."	"We need to hurry, come on." (time pressure)
"You can start with the blue one or the green one."	"Let's see if you can do this one." (perceived evaluation)
"Looks like a big feeling showed up. I'll wait with you."	"You're okay, it's not a big deal." (dismisses the state)
"Want to push the wall / squeeze the ball first?"	Re-issuing the same instruction louder or faster
"We can make this one smaller. Just the first step."	Removing the preferred item to force the transition

PROGRESS INDICATORS

How you will know it is working

Concrete, observable markers to track across the engagement. Re-rate against the early baseline.

- ✓ Shutdowns per session drop 50%+ from the Sessions 1-2 baseline.
- ✓ Time-to-re-engagement after a wobble falls below ~2 minutes.
- ✓ Riley tolerates a previewed transition with a bounded choice without a full shutdown.
- ✓ Riley orients to the visual schedule or first-then board unprompted.
- ✓ At least one novel task per session is attempted at its first step.
- ✓ Early-warning signs (slowed responses) are catchable before a full shutdown on most transitions.
- ✓ Caregiver reports the same previewing strategy reducing shutdowns at home (generalization).

SESSION LOG

Track what the checklist changes

One row per weekly visit — photocopy freely, or snap a photo for the caregiver handoff. "State at open": calm / wobble / shutdown.

DATE	STATE AT OPEN	WHAT WORKED	FLAG FOR HOME

PRESENTING PROFILE

What is happening, clinically

Presenting pattern
Transition-driven shutdowns - Riley disengages, goes quiet, and becomes physically still when moved off a preferred activity onto a non-preferred or novel task.

The Clinical Picture

Onset & course Reported by family for ~2 years; intensified after a mid-year classroom change. Most pronounced when a transition is unsignaled or time-pressured.	**Frequency** Several times per session is expected early; family reports daily occurrence at home around task changes.
Observable signs Verbal output drops to near-zero. Body stills or curls inward. Averted gaze. May grip a preferred object. Not oppositional behavior - a regulation shutdown under perceived demand.	**Clinical inference** Best understood as an anxiety-driven protective response to uncertainty, not refusal. The shutdown buys predictability. Pushing harder extends it; restoring predictability shortens it.
Antecedents / triggers Unsignaled transitions; novel materials introduced without preview; perceived evaluation ('let's see if you can'); sensory load late in session.	**Protective conditions** Visual or verbal previewing; a first-then structure; familiar materials; a brief movement/proprioceptive input before the demand; a predictable session opening ritual.

SOURCE DATA

The questionnaire answers behind this report

Every recommendation in this report traces to one of these answers, reframed through a clinical lens. In production these come from the family's responses plus the provider's engagement information form (last four rows).

Q. Child's first name
A. Riley

Q. Last initial
A. T.

Q. Age
A. 9

Q. Pronouns
A. they/them

Q. Diagnoses (sample)
A. Autism / ASD Level 1; Generalized Anxiety

Q. Communication style
A. Verbal in comfortable settings; verbal output drops sharply under perceived demand or uncertainty.

Q. The specific challenge being addressed
A. Shuts down (goes quiet, body stills, disengages) when moved off a preferred activity onto a non-preferred or novel one.

Q. When it started
A. About 2 years ago; intensified after a mid-year classroom change.

Q. How often
A. Daily at home around task changes; expected several times per early session.

Q. What it looks like
A. Near-zero verbal output, body stilling or curling inward, averted gaze, gripping a preferred object.

Q. Caregiver's read on the feeling underneath
A. Overwhelmed by not knowing what's next; the stillness is protective, not defiant.

Q. What seems to trigger it
A. Unsignaled transitions, time pressure, novel materials with no preview, feeling evaluated.

Q. What helps / when it does NOT happen
A. Previewing what's next, first-then structure, familiar materials, a movement break before the task, a predictable routine.

Q. Phrases that work at home
A. "First ___, then ___." "In two minutes we'll switch - here's what's next."

Q. Phrases to avoid
A. "Hurry up." "You're okay, it's not a big deal." Repeating the instruction louder.

Q. Provider service type (provider-answered)
A. Pediatric Occupational Therapy

Q. Session format & length (provider-answered)
A. 1:1 in-clinic, 45 min weekly

Q. Setting (provider-answered)
A. Outpatient clinic - sensory gym + table room

Q. Engagement goal (provider-answered)
A. Increase tolerance for non-preferred / novel tasks; reduce transition shutdowns.

IMPORTANT

Please read before using this report

Sample / demonstration only
This is a demonstration built on fictional data to show the report format. It does not describe a real child.

Not medical advice or diagnosis
Reports of this type are an informational resource, not a clinical assessment, diagnosis, or treatment plan, and do not replace the professional judgment of a licensed provider.

Provider judgment governs
A licensed provider should adapt every recommendation to the individual client and their own scope of practice. Strategies that help one child may not suit another.

Sponsored Access
A practice can sponsor a family's report. The family's relationship is with Pieces of Perception: they complete the questionnaire themselves, receive their reports directly, and choose whether to share a copy with the practice. The report names the sponsoring practice; Pieces of Perception remains the author.